

Doxymycin® Capsule 100mg

Doxymycin is the brand of Doxycycline Hydrochloride, synthetically derived from Oxytetracycline, and the chemical name of this light-yellow crystalline powder is alpha-6-deoxy-5-oxytetracycline. Doxymycin is a broad-spectrum antibiotic and has been shown to be active against a wide range of Gram-positive and Gram-negative organisms. It is primarily bacteriostatic and is thought to exert its antimicrobial effect by the inhibition of protein synthesis.

Ingredient(s):

Each capsule contains:

Doxycycline Hydrochloride 100mg(potency)

Pharmacodynamics:

- Doxymycin is active against a wide range of gram-positive and gram-negative organisms. The drugs in the tetracycline class have closely similar antimicrobial spectra and cross resistance among them is common. Microorganism may be considered highly sensitive if the M.I.C.(minimum inhibitory concentration) is 0.5 to 2.0mcg/ml and slightly sensitive if the M. I. C. is 2.0 to 5.0mcg/ml.
- Doxymycin is readily absorbed and is bound to plasma protein in varying degree. It is concentrated by the liver in the bile, and excreted in the urine and feces at high concentrations and is a biologically active form.
- Doxymycin is virtually completely absorbed after oral administration. Following a 200mg dose, normal adult volunteers averaged peak serum level of 2.6mcg/ml of Doxyline at 2 hours may decrease to 1.45mcg/ml at 24 hours. Excretion of Doxyline by the kidney is about 40 % / 72 hours in individual with normal function (creatinine clearance about 75ml/min). This percentage excretion may fall as low as 1 to 5 % / 72 hours in individual with severe renal insufficiency (creatinine clearance below 10ml/min).
- Doxymycin is indicated in infection caused by the following microorganism : Rickettsiae, Mycoplasma pneumoniae and the following gram-negative microbes (Haemophilus ducreyi, Yersinia pestis, Pasteurella tularensis, Bartonella bacilliformis, Bacteroides species, Vibrio cholerae, Campylobacter fetus and Brucella species.) When penicillin is contraindicated, Doxyline is an alternative drug in the treatment of infections due to Treponema pallidum and Treponema pertenue, Listeria monocytogenes, Clostridium species, Bacillus anthracis, Fusobacterium fusiforme and Actinomyces species.

Pharmacokinetics:

Metabolized in the liver. Half-life = 18 ~ 22 hours. Excretion: renal.

Indication(s):

Doxymycin has been found effective in the treatment of a variety of infections caused by susceptible strains of Gram-positive and Gram-negative bacteria such as pneumonia and other lower respiratory tract infections, chronic bronchitis, sinusitis, urinary tract infections, ophthalmic infections, sexually transmitted diseases and acne vulgaris.

Dosage and Administration:

Adults:-

For treatment of acute infection 200mg on the first day of treatment (administered as a single dose or divided into 2 equal doses with a 12 - hour interval) followed by a maintenance dose of 100mg/day. In the management of more severe infections (particularly chronic infections of the urinary tract) 200mg daily should be given throughout the treatment period.

Children:-

For children weighing 50kg or less is 4mg/kg of body weight on the first day of treatment (given as a single dose or divided into 2 equal doses with a 12 - hour interval) followed by 2mg/kg of body weight on subsequent days. For more severe infections up to 4mg/kg of body weight may be used daily. For children over 50kg, the usual adults dose should be used.

Acne Vulgaris:-

50-100mg daily for up to 12 weeks.

Acute Gonococcal Anterior Urethritis in male:-

A single dose of 300mg or 100mg twice daily for 2 or 4 days.

Acute Gonococcal Infection in adult female:-

100mg twice daily until cure is effected.

Primary and secondary Syphilis:-

300mg daily in divided doses for at least 10 days.

Uncomplicated Urethral, Endocervical or Rectal infection in adults caused by Chlamydia trachomatis:-

100mg twice daily for 7 days.

Route of administration:

Oral.

Contraindication(s):

In patients known to be or suspected of being hypersensitive to tetracycline.

Precaution(s) / Warning(s):

Some patients with spirochete infections may experience a Jarisch-Herxheimer reaction shortly after doxycycline treatment is started. Patients should be reassured that this is a usually self-limiting consequence of antibiotic treatment of spirochete infections.

- Exceeding the recommended dosage of Doxymycin may result in an increased incidence of side effects.
- The use of Doxymycin during tooth development (last half of pregnancy, infancy and childhood to the age of 8 years) may cause permanent discoloration of the teeth (yellow-gray-brown). This adverse reaction is more common during long-term use of the drug but has been observed following repeated short-term courses.
- As with other antibiotics, use of Doxymycin may result in overgrowth of non-susceptible organisms, including fungi. If superinfection occurs, appropriate therapy should be instituted.
- In venereal disease when co-existent syphilis is suspected, a dark field examination should be done before treatment is started and the blood serology should be repeated monthly for at least 4 months.
- Because Doxymycin has been shown to depress plasma prothrombin activity, patients who are on

anticoagulant therapy may require downward adjustment of their anticoagulant dosage.

6. In long-term therapy, periodic laboratory evaluation of organ systems, including hematopoietic, renal and hepatic studies should be performed.
7. Since bacteriostatic drugs may interfere with the bactericidal action of penicillin, it is advisable to avoid giving Doxymycin in conjunction with penicillin.
8. May cause a yellow to brown discoloration of the teeth and enamel, hypoplasia in the developing fetus or child and therefore, Doxycycline should only be administered if considered essential to pregnant or lactating women and children under twelve years of age.
9. Can cause photosensitivity and allergic skin reactions.
10. It should be administered with caution to patients with hepatic and renal impairment or those receiving potentially hepatotoxic drugs.
11. Absorption of tetracycline is impaired by antacids containing aluminium, calcium, magnesium or zinc and iron containing preparation.
12. Barbiturates, carbamazepine and phenytoin decreased the half-life of doxymycin.
13. The concurrent use of tetracycline and methoxyflurane has been reported to result in fatal renal toxicity.

Interaction with other medicaments:

Absorption decreased by antacids, Calcium, Iron, Magnesium salts. May interfere with bacterial action of penicillin.

Side Effect(s) / Adverse Reaction(s):

1. Gastrointestinal: Anorexia, nausea, vomiting, diarrhea, glossitis, dysphagia, enterocolitis, inflammatory lesions in the anogenital region.
2. Skin and subcutaneous tissue disorders: Maculopapular and erythematous rashes. Frequency 'rare': Fixed eruption
3. Hypersensitivity reactions: Anaphylaxis, anaphylactoid purpura, pericarditis and exacerbation of systemic lupus erythematosus.
4. Blood: Haemolytic anemia, thrombocytopenia, neutropenia, eosinophilia.
5. Renal toxicity: Rise of blood urea has been reported with tetracycline and is apparently dose related.
6. Bulging fontanelles in infants and benign intracranial hypertension in adults have been reported in some individual receiving full therapeutic dosages. These conditions disappeared rapidly when the drug was discontinued.
7. Immune system disorders.
Frequency not known: Jarisch-Herxheimer reaction (see Section Warnings and Precautions)

Pregnancy and Lactation:

May cause a yellow to brown discoloration of the teeth and enamel, hypoplasia in the developing fetus or child and therefore, Doxycycline should only be administered if considered essential to pregnant or lactating women and children under twelve years of age.

Symptoms and Treatment for Overdosage and Antidote(s):

There is no specific antidote for Doxymycin overdose, therefore management of the patient should consist of symptomatic and supportive therapy.

Shelf-Life:

3 years from the date of manufacture.

Storage Condition(s):

Store at temperature below 30°C. Protect from light and moisture.

Product Description:

A #2 dark green hard gelatin capsule containing yellow powder.

Packing(s):

Blister packing of 10's x 10, 10's x 50 and 10's x 100.



Manufacturer and Product Registration Holder:
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